Guidelines for School Reintegration following Psychiatric Hospitalization for Suicide-Related Crises

Reccomendations for Hospitals, Schools, and Families



University of North Carolina at Chapel Hill
School of Education

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ACKNOWLEDGEMENTS

We would like to thank the families and professionals who donated their time.

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Funding:

This project was supported by Grant SRG-0-093-17 awarded to Marisa Marraccini from the American Foundation for Suicide Prevention. The project described was also supported by the National Center for Advancing Translational Sciences (NCATS), National Institutes of Health, through Grant Award Number UL1TR002489. Marisa Marraccini's effort was supported by the National Institute of Mental Health (K23MH122775; L30MH117655; Marraccini). The efforts of and/or resources used by Amanda Tow, Juliana Vanderburg, and Christina Cruz were also supported by the National Center for Advancing Translational Sciences, National Institutes of Health, through Grant KL2TR002490. The content is solely the responsibility of the authors and does not necessarily represent the official views of the American Foundation for Suicide Prevention or NIH.

INTRODUCTION

How to Use these Guidelines

These guidelines were created to provide recommendations and considerations for hospitals, schools, and families in supporting youth returning to schools. They are based on existing research and a North Carolina based study that explored the perspectives of youth previously hospitalized for a suicide-related crisis and their caregivers, school professionals, and hospital professionals.

Always follow your school or hospital's protocols, considering these recommendations as a way to enhance existing care.

Brief Overview of Prevalence & Risk for Suicide

- Risk for suicide is complex, with biological, psychological, and environmental risk factors.
- Rates of suicide-related thoughts and behaviors have increased from 2019-2019 based on adolescent self-report (CDC, 2020).
- Annual percentage visits for suicidal ideation and attempts nearly doubled from 2008 to 2015.
- Hospitalization for suicide attempts increased in the winter of 2021, especially for girls (Yard et al., 2021)

Suicide is the 2nd leading cause of death among adolescents (CDC, 2022)

Who is psychiatrically hospitalized?

- Hospitalized youth represent a diverse group of students, including students of all racial, ethnic, sexual, and gender identities, cultural and linguistic backgrounds, and socio-economic status.
- Risk factors for suicide and suicide-related thoughts and behaviors and how symptoms related to co-occurring conditions present may vary according to an individual's cultural, linguistic, gender, racial, ethnic, and sexual identity background.

What is psychiatric hospitalization?

- Inpatient care may involve different treatment modalities that address safety, stabilization, and psychopharmatherapy.
- Adolescents may be hospitalized on a child, adolescent, or adult unit, depending on their age and hospital availability.
- Treatment varies across hospitals.
- Families have varying experiences depending on their situation and the specific hospital - but even in the best situations, the experience is disruptive and difficult.
- Discharge usually involves the development of a safety plan intervention and plan for ongoing care and treatment.
- Youth discharged from the hospital usually remain at risk for suicide.

What are HIPAA and FERPA?



HIPAA – Adhering to HIPAA

The Health Insurance Portability and Accountability Act of 1996 (HIPAA) is a federal law that includes the HIPAA Privacy Rule. You must follow these standards, which address the use and disclosure of individuals' protected health information (PHI). For more information, visit the CDC website.



FERPA – Adhering to FERPA

The Family Educational Rights and Privacy Act (FERPA) is a federal law that "protects the privacy of student education records" and applies to schools receiving funds from the U.S. Department of Education. For more information, visit the U.S. Department of Education website.

UNIVERSAL CONSIDERATIONS

There are several universal strategies schools can use that may benefit all students, and may also help set the stage to better support students experiencing mental health crises in the future.

Establish clear procedures regarding school reintegration and disseminate to school professionals, families, and clinicians.

- Student and family handbook should describe how to involve schools in the case of psychiatric hospitalization
- School website should identify who families and clinicians should contact to provide information about mental health crises and concerns.
- Orientations for students and families should include information about processes.

Provide psychoeducation about suicide-related risk and recovery from mental health crises to school community.

- Attitudes and beliefs of school professionals about suicide can influence student experience upon return.
- Professional development can address risk factors for suicide, warning signs about suicide-related risk, and trajectory of recovery following a crisis.

Foster a positive school psychosocial climate.

Foster school-family partnerships.

Foster school-community partnerships and establish culturally grounded referral networks.

DURING HOSPITALIZATION

Preparing for Re-Entry

Hospitals

1. **Consider Return to School Throughout Hospitalization.** Integrate discussions around school-related stressors and supports with students and families into hospital treatment and discharge planning

2. **Discuss Information Sharing with Families.** Discuss if families would like to share information with school, considering the following:

- **Attendance Policies**: Does the school require documentation of hospitalization to return?
- **IEP/504 Plan**: Does the returning student have an IEP or 504 that needs to be updated? Does the returning student have a disorder or concern that merits a request for an IEP or 504 evaluation?
- **Re-Entry Meeting**: Because many schools do not automatically hold a re-entry meeting for returning students, does the family know how to request a re-entry meeting to identify a re-entry plan?
- **Informal Supports**: Would the student benefit from any informal supports that the school can provide?
- **Comfort**: Are there individuals at the school that the patient and family trust to initiate the processes with?
- The student's school counselor may be appropriate for families

Hospitals continued…

3. **Discharge Summary for Schools.** Provide a summary and include recommendations for relevant school-related supports and services that can be shared with the school if families provide permission:

- **Summary.** Consider identifying the following information for schools to integrate into their re-entry planning process:
  - School-related stressors and triggers that can inform school supports for returning students
  - Any diagnoses that could be used for formal supports requiring an IEP or 504 plan can be clearly identified
  - Reasons for hospitalization if applicable to school settings
  - Medications that may impact student's functioning in school or require medication management in school
  - Coping strategies that can be used by students in school
- **Psychoeducation on Hospitalization and Re-Entry Protocols.** Provide an overview of goals related to psychiatric hospitalization, and information about general procedures used by schools to facilitate re-entry
- **Safety Planning.** Integrate school environment into safety planning procedures (e.g., identify trusted adults in school in addition to those available at home, identify coping strategies that can be appropriate for classroom settings) and consider sharing with school

4. **Recommendations for Schools.** Provide recommendations for schools supporting returning student tailored to patient's needs

- **Recommendations for Modifications.** Consider the following modifications available in most schools for returning students, tailored to each student's needs. Also consider the timing of modifications (are these modifications students may need long-term or modifications that should be reassessed regularly?)
  - Gradual Return to School
  - Excused Absences
  - Pass to Attend School Late or Leave School Early
  - Consideration of Missing Work Forgiveness
  - Consideration of Extended Deadlines or Reduced Assignments/Work Load*
  - Opportunity to Retake Tests*
  - Opportunity to Take Tests in a Quiet Location*
  - Extended Time Limits for Tests*

Hospitals continued...

- **Recommendations for Interventions.** Consider the following interventions available in most schools for returning students, tailored to each student's needs. Also consider the timing of intervention (are these interventions students may need long-term or modifications that should be reassessed regularly?)
  - Support with Work Completion/Time Management
  - Tutoring
  - Access to a Transition Classroom within School (i.e., a separate space for academic and/or social-emotional support)
  - Check-in/Check-out (i.e., regularly occurring meetings with an identified school member to check in about social, emotional, and academic needs and progress)
  - Individual counseling

Consider the following interventions that are only available in some schools, tailored to each student's needs:

- Self-Monitoring Instruction**
- Group Counseling**
- Social Skills Groups**
- Peer or Adult Mentoring Programs**
- School-based Mental Health **

5. **Consider Variability Across Schools.** Be careful to set appropriate expectations for families since schools may vary in the availability and willingness to provide supports and services to returning students

Although available supports and services may vary by school, * indicates a modification that may commonly require an IEP or 504 plan and ** indicates an intervention that may commonly vary according to school resource availability.

IEP = Individualized Education Plan

Preparing for Re-Entry

Schools

- Meetings with families to prepare for return
- Gestures of support to hospitalized student
- Preparation of re-entry meetings and plan
  - Collecting work/considering content missed
  - Notifying other staff (teachers)
  - Educator attitudes
  - Student attitudes

Information Sharing

Cautions

Although some individuals prefer to receive and provide as much information as possible to enhance student care, ***there are policies in place to protect individual and family rights to privacy, as well as additional concerns to consider when determining if and what to share across sites.***

Benefits

- Informed Hospital Diagnosis and Treatment
- Attendance Policies
- IEP/504 Plan
- Re-Entry Meetings
- Informal Supports
- On-going Care

Type of Information	Considerations
Medications	• Side effects • Management in school
Coping Strategies	• Limitations according to circumstances
Diagnosis	• Requirements for services in school
Reason for Hospitalization	• Relatedness to school
School-related stressors or supports	• Consider school-related experiences during hospitalization
Psychoeducation about hospitalization and youth in crisis	• Standard procedures
Recommendations for school	• Variability in schools and student needs
Safety planning	• School conext



School Questions for Families

- Who would you like me to notify/not notify?
- What would you like me to tell the people we are notifying?
- What work would you like me to collect?
- Do you want me to reach out to the hospital?
- Run through “5 minute phone call” to ask permission for this information

*They are also going to want to use their own judgement of staff to consider best methods of sharing, including staff considerations. Get a handle on attitudes of students/staff to help determine process.

School Topics for Hospitals

- Consider a point person at school for hospital contact
- Conversation about work completion expectations
- Things to consider sharing about the student
- Preparing for return, what to prepare
- What standard info is good to share (5 minute phone call)
-
- Establish communication process for this student and over long term

Addressing Suspected Hospitalizations

Sensitivity to family's wishes....

- Ask family, but be respectful.
- Adhere to standard policies.
- Validate concerns and reasoning.

Actions

- Monitor attendance and follow-up accordingly
- Provide supports and services based on observed behaviors/needs
- Address rumors

POST-HOSPITALIZATION

Considerations will vary according to timing of school year.

- [ ] Student Centered & Strengths Based
- [ ] Re-Entry Meetings
- [ ] Individuals Involved
- [ ] Re-Entry Plan
- [ ] Safety Plan

Who is Involved?

Individual/Role	Considerations
Student	Chance for advocacy/self advocacy Psychological functioning (one-on-one vs. group)
Family	Consideration of different roles
Point person	Someone who can coordinate information and meetings to support re-entry: counselor, psychologist, social worker, others?
Special Education/504	If has services or may require services, representatives for IEP or 504 may be required
Administrator	To ensure school-wide decisions/implications
Nurse	Especially if medication management or delivering social emotional supports
Student Support Staff	Individual delivering social-emotional supports (e.g., counselor, nurse, social worker, psychologist) that student may meet with
Adult Connected to Student	A person in school identified by student or family as trusted who student can reach out to (e.g., CICO)
Mental health provider	Community or school-based therapist/MH provider who can provide information about supports in school

Re-Entry Plan Elements

1. Student's needs, strengths, skills
2. School-related stressors
3. School-related supports and interventions
4. Assessments and plan for continuing or changing supports
5. Safety planning intervention
6. Identification of key individuals

Considerations if school is unaware....

- Many ways to learn about hospitalization
- Sensitivity to student and family wishes
- Re-entry plans: Better late than never!